

Interview Transcript with Dr. Lukas Meier (Berlin, Germany) Senior Pulmonologist and Critical Care Specialist, Charit University Hospital

Interviewer: Dr. Meier, thank you for joining us. Could you begin by telling us about your background and how you were involved in the pandemic response? Dr. Meier: Thank you for having me. I'm Dr. Lukas Meier, a pulmonologist and critical care specialist at Charit University Hospital in Berlin. Before the pandemic, my focus was on chronic lung diseases and intensive respiratory support. When COVID-19 hit, our department became the frontline transforming overnight into one of Germany's largest dedicated COVID intensive care units.

In the spring of 2020, my team managed some of the earliest severe COVID cases in Europe. I was part of the hospital's pandemic task force that coordinated ICU expansion, staff training, and ventilator allocation across Berlin's hospital network.

Interviewer: Germany was praised early on for its COVID response. What was that initial phase like from your perspective? Dr. Meier: The early response in 2020 was indeed structured and well-coordinated. Germany had a head start thanks to our strong laboratory network and decentralized health system. We began widespread PCR testing earlier than most European countries, which allowed us to detect clusters quickly.

In those months, we had enough ventilators and protective gear. The challenge wasn't capacity it was uncertainty. Every day, we were learning something new about the virus: how it affected oxygenation, clotting, or the heart. Those were intense weeks medicine felt both exhilarating and terrifying.

Interviewer: How did the ICU system adapt as the crisis deepened? Dr. Meier: The key was coordination. Germany's DIVI Intensive Care Registry, which tracks available ICU beds nationwide, became our lifeline. Every hospital updated capacity daily available beds, ventilators, and staff. That transparency prevented chaos.

When ICUs in Munich or Hamburg filled up, patients were airlifted to less burdened regions. We even accepted patients from Italy and France during their early surges. The solidarity within Europe was palpable, even when we were all overwhelmed.

However, as the second and third waves arrived, fatigue set in. By late 2021, staff shortages not ventilator shortages became our bottleneck.

Interviewer: How did your team handle triage when ICU demand peaked? Dr. Meier: Triage is every doctor's nightmare. Officially, Germany never adopted a national triage policy, but informal prioritization did occur. When ICU beds were scarce, we used a multidisciplinary ethics board doctors, nurses, and ethicists to guide decisions.

We evaluated factors like survival probability, comorbidities, and potential for recovery. But it never felt easy. Every patient is someone's loved one. I remember one night when we had only one ventilator left and two critical patients both in their 60s. The one with fewer

comorbidities was intubated first. The other passed away. That decision still haunts me.

Interviewer: What were the biggest medical challenges in treating severe COVID cases? Dr.

Meier: COVID-19 ARDS (Acute Respiratory Distress Syndrome) was unlike anything we had seen. Traditional ventilation strategies often failed. We relied heavily on prone positioning, deep sedation, and ECMO (extracorporeal membrane oxygenation). Charit has one of Europe's largest ECMO programs, so we became a referral hub for critical patients across Germany.

Another challenge was thromboembolic complications pulmonary embolisms, microclots in the lungs. Anticoagulation protocols changed multiple times as evidence evolved. It was a constant balance between preventing clots and avoiding bleeding.

COVID humbled medicine. It reminded us how little we sometimes understand about our own physiology under extreme stress.

Interviewer: How did you manage your team's morale through such prolonged intensity? Dr.

Meier: That was perhaps the hardest part. For months, our team worked 14-hour shifts in full PPE. The psychological weight of constant death, isolation, and uncertainty was enormous.

We introduced decompression sessions short debriefs at the end of shifts where staff could talk or simply sit in silence. Psychologists joined our rounds weekly. We also encouraged physical activity short yoga sessions or music therapy on weekends.

Some of us coped by documenting our experiences. I kept a personal log, not of medical data but of emotions frustration, fear, pride. That became my therapy.

Interviewer: Germany's vaccination campaign started strong but faced resistance later. How did that look from your vantage point? Dr. Meier: It was frustrating. In early 2021, when

vaccines became available, we were euphoric. I received my first dose in January it felt like light at the end of a tunnel. Initially, uptake was high among healthcare workers and the elderly. But as rollout expanded, misinformation spread rapidly, especially in certain regions.

Some patients came in unvaccinated and dying, still convinced COVID was exaggerated. That broke our hearts. We tried to counsel them even in ICUs, though it was often too late. Vaccine hesitancy wasn't just ignorance it was mistrust, amplified by online echo chambers.

On the positive side, Germany's data transparency allowed us to prove vaccine effectiveness quickly hospitalization rates dropped sharply among the vaccinated.

Interviewer: How did Germany's federal structure help or hinder the response? Dr. Meier:

Both. The federal system allowed local flexibility states could tailor measures to infection trends. But it also created inconsistency. Patients and even doctors were confused by changing rules from one state to another.

However, coordination between federal agencies like the Robert Koch Institute (RKI) and hospitals improved over time. The RKI's real-time data dashboards became our compass. Germany

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decentralized model ultimately worked because hospitals communicated constantly not because everything was centrally perfect, but because clinicians took initiative.

Interviewer: Did your hospital collaborate internationally during the pandemic? Dr. Meier: Extensively. Charit has collaborations with hospitals in Italy, the UK, and Israel. During the first wave, we shared ventilator protocols and mortality data weekly with colleagues abroad. Science became borderless we learned as much from WhatsApp chats with Italian intensivists as from formal studies.

Later, we participated in European clinical trials testing anticoagulant regimens and steroid dosing. That collaboration accelerated learning. COVID may have isolated people physically, but it united scientists intellectually.

Interviewer: Were there any ethical or personal moments that stand out to you? Dr. Meier: Many. I remember an elderly patient, Frau M ller, who refused intubation. She said, If I must die, I d rather see faces, not machines. We placed her by the window and played her favorite classical music. She passed peacefully. Moments like that reminded us that dignity is as vital as survival.

Another was watching a recovered patient walk back into the ICU to thank the nurses who had cared for him. He brought chocolate for everyone. Those small gestures meant everything after months of relentless struggle.

Interviewer: Looking back, what do you think Germany did particularly well, and what could have been better? Dr. Meier: We did well in testing, ICU coordination, and scientific rigor. Our early laboratory preparedness saved lives. But we underestimated pandemic fatigue how public compliance erodes over time. Communication should have been more emotionally intelligent, not just data-driven.

We also learned that resilience depends on redundancy more staff, not just machines. Future preparedness must focus on training and retaining nurses and respiratory therapists, not merely purchasing ventilators.

Interviewer: What do you personally take away from this experience? Dr. Meier: Perspective. The pandemic stripped away illusions of control. We like to think medicine conquers uncertainty, but in truth, we coexist with it. I now approach patients with greater patience and empathy every breath feels sacred after watching so many struggle for it.

It also reinforced my belief in teamwork. No one survives such a crisis alone not patients, not doctors, not societies. The virus tested our systems, but it also revealed our shared humanity.

Interviewer: That s deeply moving. Any final thoughts for future generations of healthcare professionals? Dr. Meier: Yes. Prepare not just your mind, but your heart. Science will guide

you, but compassion will sustain you. Pandemics will come and go; what matters is how we treat one another in those moments of chaos.

Medicine is, ultimately, an act of service not of certainty, but of courage.

Interviewer: Thank you, Dr. Meier. Dr. Meier: Thank you. And may we never forget the lessons written in those ICU corridors.

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